

TRACHEOSTOMY

PAEDIATRIC · BEDHEAD SIGN

This child breathes through a tracheostomy tube. Oxygen goes to the **FACE** and the **STOMA** from the first second.

UPPER AIRWAY PATENT ☐

NECK BREATHER — use the RED sign ☐

Name

Hospital no.

Date inserted

Tube make / type

Size (internal diameter)

Cuffed Y / N

Inner cannula Y / N

Suction catheter (Fr)

Suction depth (cm)

Stay sutures Y / N

Consultant / bleep

IN AN EMERGENCY

1. Call for help. **Oxygen to the FACE and the STOMA.**
2. **Look at the tube — is it still in the stoma?** If it has come out, reinsert it. That is the whole problem.
3. Remove what is removable — HME, speaking valve, cap, inner cannula.
4. Pass a suction catheter. If it passes, suction. If it will not pass the tube is not an airway — **remove it.**
5. Still not breathing — **upper airway FIRST:** bag-mask over mouth and nose while occluding the stoma. Ventilate at the stoma only if the upper airway is not patent or that fails.

TWO TUBES, ALWAYS

Same size + half-size smaller — both with obturators.

- Never force. Reposition and retry the same size, then the half-size once, then stop and ventilate.
- The obturator comes out the instant the tube is seated.

Follow the paediatric tracheostomy (green) emergency algorithm. National Tracheostomy Safety Project; Doherty C, et al. *Anaesthesia* 2018;73:1400–17. doi:10.1111/anae.14307 · Check this sign at every handover.

NECK BREATHER

LARYNGECTOMY · BEDHEAD SIGN

There is **NO** connection between the mouth and the lungs. **ALL** oxygen and **ALL** ventilation go to the STOMA.

Name

Hospital no.

Date inserted

Tube make / type

Size (internal diameter)

Cuffed Y / N

Inner cannula Y / N

Suction catheter (Fr)

Suction depth (cm)

Reason for no upper airway

Consultant / bleep

IN AN EMERGENCY

1. Call for help. **Oxygen to the STOMA. Do not waste time on the face.**
2. **Is the tube still in the stoma?** If it has come out, reinsert it — that is the whole problem.
3. Remove what is removable — HME, valve, cap, inner cannula.
4. Pass a suction catheter. If it will not pass, remove the tube.
5. Ventilate at the stoma: a paediatric face mask applied over the stoma, or a small supraglottic device over the stoma.
6. Oral intubation will **not** work. Any rescue is at the neck.

DO NOT

- Do not give oxygen or ventilation by mouth or nose.
- Do not force. Reposition the neck and retry the same size, then the half-size once.
- Do not dilate the stoma blindly. That makes a false passage, not an airway.
- **Two tubes travel with this child** — same size and half-size smaller, both with obturators.